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Medicine

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Case Notes Study 1

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WRITING SUB-TES: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a junior physician working at Cow Bay Primary Health Clinic, 69 Tea Tree Rd, Diwan QLD 4873, Australia, and Mrs. Julia Smith has presented in active labor and baby was delivered successfully.

PATIENT DETAILS:

Name: Julia Smith
Age: 21
Husband: Mason Smith

Past history:
- No records available, from Cyprus, visiting in-laws in Australia
- Used to get monthly visits from local midwife
- No hospital visits
- No vaccinations
- Allergies unknown

Family History:
- Mother diabetic
- Father hypertensive, had stroke 2 years back
- 2 elder sisters, eldest diabetic
- 4 brothers, 2 hypertensive and diabetic
- 1 had inguinal hernia

05-12-2021
SUBJECTIVE:
- G1 P0
- 30 weeks of gestation
- Vaginal bleeding small amount
- Burst water bag 6 hrs ago
- Severe abdominal and back pain, episodic

OBJECTIVE:
08:04 A.M
- Bloody show occurred
- Ruptured amniotic membranes
- Strong uterine contractions 2 mins apart
- Cervical effacement 90%

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- Head engaged, station +1
- Cervical dilation 6 cm
- Fetal heart sounds audible
- Maternal blood pressure 140/90 mmHg
- Maternal heart rate 98/min

PLAN:
- Deliver the baby since patient in active labor
- I.V. hydralazine hydrochloride 5-10 mg every 15-20 minutes until BP under control
- No epidural (patient refused)
- Close monitoring, maternal vitals, progress of labor
- Oxygen if needed

08:40 A.M
- 100% cervical effacement
- Fetal station +4
- Strong, frequent uterine contractions
- Stable CTG
- Maternal BP 130/90 mmHg
- Heart rate 80/min

PLAN
- Stop hydralazine
- Support labor progression, deliver baby

09:10 A.M
- Delivered baby boy
- Mother stable
- No vaginal lacerations

Baby's Nursing notes

09:11 A.M
- APGAR 5
- Weight: 4.1kg
- Not crying, cyanosed
- Massaged and back slapped, crying started
- Kept on warmer
- Nasal suctioning done

09:16 A.M
- APGAR 7
- Hypothermic & cyanosed

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PLAN: Immediate transfer of both mother and baby to hospital with NICU for:

- Neonatal resuscitation and premature neonatal care
- Complete examination by pediatrician
- Maternal risk factor management by obstetrician
- Counselling for next pregnancies

Writing Task

Using the information in the case notes, write a letter of transfer to Queensland Royal Hospital where Mrs. Smith and her newborn baby can be monitored and cared for by an obstetrician and pediatrician respectively. Address the letter to the Admission Officer.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 2

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Assume today's date is 19/01/2022.
You are a GP practising at Collin's Clinical Center, and Mrs. Kathy Shawn is a new patient.

PATIENT DETAILS:
Name: Kathy Jonnas Shawn
Age: 26 years
Marital Status: Married (04/04/2018)
Occupation: Sales clerk
Husband: Jonnas Shawn
Husband's Occupation: Civil engineer

Presenting Complaints (started 24 hours ago):
- G2 P0
- In 20th week of gestation
- Dizziness, headache, nausea, vomiting
- Blurred vision
- Abdominal pain
- Vaginal spotting

Past Antenatal Visit:
- 5th week of gestation (pregnancy confirmation - ultrasound and urine PT)
  Counselled, aspirin 100 mg daily
  Weekly visits advised
- Lost to follow-up

Past Medical History:
- Intermittent Asthma (10/07/2014), Albuterol inhaler SOS

Past Surgical History:
- Appendectomy (14/05/2012)

Past Gynecological and Obstetric History:
- Pelvic inflammatory disease (14/07/2013), completed 2 week antibiotic course
- First pregnancy ended in miscarriage,12 week gestation (02/07/2019) secondary to:
  - Pre-eclampsia
    Avg. BP throughout pregnancy 150/90 mmHg, pitting edema face and legs, proteinuria
    Previous GP advised lifestyle modification and aspirin 100mg daily

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- Antiphospholipid syndrome (07/10/2019)
  Diagnosed (after miscarriage) with ELISA positive for anti-cardiolipin antibodies
  No treatment since asymptomatic status, counselling about next pregnancy

Allergies:
- Dust and pollen
- Penicillin

Social History:
- Studied till middle school
- Smoker (10 cigs daily)
- Occasional crack and alcohol intake

Family History:
- Mother diabetic, hypertensive
- 2 half sisters, unknown medical history
- Father died of pancreatic cancer 24 years back

Physical Examination:
- Confused but oriented in person, place, time
- BMI 31 kg/m2
- BP 152/90 mmHg
- Pulse 90/ min
- RR 22/min
- B/L pedal edema pitting type
- Facial puffiness, periorbital edema
- Mild RUQ abdominal tenderness

Obstetric Examination:
- Fundal height 23cm
- Tender to deep palpation
- Fetal lie oblique
- Fetal heart sounds audible but faint

Provisional Diagnosis: Pre-eclampsia

Plan
- Urgently refer patient to obstetrician for close monitoring & adequate blood pressure control to prevent progression to eclampsia

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Writing Task

Using the information in the case notes, write a letter of referral to Dr Anita Arora, an obstetrician at Melbourne Maternity Centre, East Melbourne 3007, for urgent management.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 3

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WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a station house officer at Royal Adelaide Hospital and have been managing Mr. Peter Berry for the past 6 months.

Hospital: Royal Adelaide Hospital, Port Rd, Adelaide SA 5000, Australia

Date of Admission: 26/06/2021

Diagnosis: Traumatic head injury (RTA)

Patient Details:
Name: Mr. Peter Berry
DOB: 26/03/1991 (30 yo)
Marital status: Single
Occupation: Accountant
Next of Kin: Amelia Andy (55, mother)

Social Background:
- Smokes 9 cigs daily
- Moderate alcohol consumption
- Frequent overspeeding fines
- Mother OT scrub nurse
- Father died of pancreatic cancer 4 years back

26/06/2021
- Brought to emergency after severe automobile collision
- Primary and secondary trauma survey, FAST, CT head done
- Subdural hematoma, right femur fracture, initial resuscitation (ABCDE and IV 0.9% NaCl)

28/06/2021
- Patient hemodynamically stable (bp 106/78 mmHg, pulse 90/min, afebrile, RR 22/min)
- Craniotomy & repair of femur fracture
- Patient vitally stable
- Not awaken from surgery

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01/07/2021
- Patient still non-responsive, vital functions normal (bp 100/80 mmHg, pulse 94/min, afebrile, RR 26/min)
- IV mannitol, IV broad spectrum antibiotics, IV fluids administered, supportive care, ICU monitoring

07/09/2021
- Coma reversal, GCS 9, problems with speech, delirious state
- Baseline investigations sent, electrolyte abnormalities corrected, pain medications

11/10/2021
- ↑ GCS 13, oriented, alert, conscious
- Problems with speech and mobility
- Physical rehab & speech therapy started

24/12/2021
- Social withdrawal, hopelessness, suicidal ideation
- Difficulty with speech reduced but need wheelchair assistance
- Mother requests transfer to mental health and rehab facility for better care

Plan:
- Continue speech therapy and physical rehab
- Psych assessment for depression
- Counselling for social behavior (speeding, alcohol, smoking)

Writing Task

Using the information in the case notes, you need to write a transfer letter to the director of Mary Curie Physical and Mental Health Rehabilitation Center, Mr. Edward Collins, to provide further care for Mr. Peter Berry.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long

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Case Notes Study 4

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a venereal disease specialist working at the Department of Communicable Diseases, Australia. Today, you examined Mr. Henry Mathews, referred by his GP, Dr. Kumar.

Patient details:
Name: Henry Mathews
Age: 24 years
Occupation: Local club owner
Marital status: Single
Address: 230 Spencer Street, West Melbourne
Next of kin: Bethany Mathews (28, sister)

Past medical history:
- ADHD (20th September 2002)
- Mild meningococcal meningitis (12th August 2014)
- Essential hypertension (10th May 2017)

Past surgical history:
- Laparoscopic varicocele repair (25th April 2017)

Family history:
- Father died of alcoholic liver disease 8 years back, was abusive, had multiple extramarital affairs
- Mother diabetic, caring, volunteer at local community service center
- 3 siblings, patient is youngest.
- Lives with eldest sister who is homemaker
- Twin brother is engineer in China

Personal history:
- Heavy drinker, started after first break-up (July 2014)
- 10-12 cigarettes per day for 15 years
- Occasional one-night stands, new relationship of 14 days
- No previous history of STDs

Medications:
- Hydrochlorothiazide 25 mg OD
- Captopril 12.5 mg PRN (BP >140/90)
- Atorvastatin 10 mg HS

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Allergies:
- Penicillin
- Metals

Reason for referral:
- Complicated STD

25/01/2022
Presenting complaints:
- Rash on genitals, extremities, and trunk
- Fever, sore throat, headache, malaise
- Joint pain

Examination findings:
- BP: 130/90 mmHgm Temp. 101F, Pulse: 90/min, RR: 18/min
- Maculopapular rash all over trunk and extremities including palms and soles (rash pinkish, symmetrical, non-itchy)
- Flat, velvety, painless lesions on genitals
- Swollen painful, red knuckles

Test results:
- CBC: Hb 10.5 g/dl, Hematocrit 31.5, Wbc 14,000/ mm3, Neutrophils 86%, Platelets 103,000/ mm3
- Rh Factor: neg
- HIV ELISA: neg
- Hep B & C serology: neg
- Monospot: neg
- RPR: positive
- Dark field microscopy of condylomata: Treponema pallidum bacteria visible
- Treponema pallidum particle agglutination (TPPA) test: positive

Diagnosis: Secondary syphilis

Treatment:
- Tab. Doxycycline 100 mg BID for 14 days
- Tab. Paracetamol 1000 mg QID for fever, SOS for joint pain
- Avoid sexual contact until rash heals completely
- Report any severe, new-onset symptoms immediately

Plan:
- Report local health department
- Update GP for
  - Proper follow-up
  - Safe sexual practise counselling (reinforcement)
  - Treatment compliance & long-term complications guidance
  - Inform and assess all possible contacts maintaining confidentiality
  - Vaccination (hep B, C)

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Writing Task

Using the information in the case notes, write an update letter regarding Mr. Mathews' condition to his general practitioner, Dr Aneel Kumar working at Melbourne City Clinic, 98 Brick Road, West Melbourne 3007.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 5

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Mrs. Ruth James was brought to ED of a hospital in which you are Emergency Registrar.

Patient's Name: Mrs. Ruth James
DOB: 12/02/1986
Address: 256 Park Avenue, NY
Social Background: Married, works in an IT company as programmer

Family History:
- Father diabetic
- Mother & sister asthmatic

Medical History:
- Asthma since 20 years of age (Salmeterol 50mcg PO inhaler BID)
- No known drug allergies

25/07/2021
Subjective
- Blunt injury isolated to face following fall from ladder when cleaning the ceiling
- loss of consciousness (5 mins)
- Oriented and conscious on admission

Objective:
- Dish-face deformity, multiple lacerations
- Dropped upper jaw
- Profuse bleeding from nose and mouth
- Both nares obstructed, breathing from mouth with difficulty
- Periorbital oedema
- Vital Signs – BP- 130/100, HR- 86, RR- 12, Sats- 80 %
- Eye movements and vision normal
- Facial examination
  - 3 cm laceration nasal bridge
  - 4cm laceration upper lip extending over face
  - Tenderness in frontal and maxillary sinus area bilaterally
  - Orbital margin tender
  - Floating maxilla

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- Intraoral examination
  - Malocclusion
  - Displaced both central incisors
  - Mandibular and ear examination: Normal bilaterally

Investigations:
- CT Head (frontal contusion and haemorrhage)
- CT Facial bones (fractured pterygoid plate & orbital floor)
- GCS (normal)
- CBC, Coagulation and electrolytes (normal)

Assessment:
- Maxillary Fracture

Plan
- Nasotracheal intubation over a flexible bronchoscope
- Control bleeding
- Ringer Lactate Infusion
- Augmentin 1.2g I.V., every 8 hourly
- Urgent referral to surgeon - Restoration of functional occlusion, reconstruction of stable bony facial contour, intermaxillary fixation for stabilization followed by rigid fixation of maxilla

Writing Task

Write a letter to Dr. Thomas John Perkins, a maxillofacial surgeon in New York Medical Center, Queens, NY, for urgent surgical management of the patient.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 6

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a cardiologist at the Coronary Care Unit, Alaska Regional Hospital, United States, and the patient has been admitted to the CCU.

Today's Date: 18/11/21

Date of Admission: 14/11/21

Patient Details:
Name: Sabrina Jagosh
Gender: Female
Age: 53 years

Social History:
- Widowed, Retired salesperson
- Has 1 adult son - lives abroad
- Smoking - 14/day
- Alcohol - 17 units/week

Medical History:
- Has hyperlipidaemia and HTN -10 years- takes Rovista 20 mg HS and Atacand 10 mg OD
- Family medical history of HTN and T2DM
- Allergies: Penicillin and dust

14/11/21
Subjective
- Reported to ED
- C/o left sided chest heaviness, pain radiating to left arm, jaw and epigastric region, heavy perspiration, vomiting, and air hunger (since 15 mins)

Objective
- Abdomen non-tender
- BP- 126/95, Pulse- 112/min, RR - 25 BMP, O2 Sat - 91%, Temp.- 37.4 C

Initial ED diagnosis: ? Myocardial infarction

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Management:
- Applied O2 10L via face mask
- Administered morphine IV
- Diagnostic workup
- ECG-prominent Q waves, T wave inversion
- U/S Abd-NAD
- Shifted to CCU immediately, vomited while transfer

15/11/21
Subjective
- Chest pain increased

Objective
- Speech- slurred
- GCS 15/15

Investigations:
- ECG repeated- ST elevation in leads I, aVL, V5, and V6

Diagnosis: Anterior wall MI

Management Plan:
- Started O2 15 L/min, TPA therapy clopidogrel, aspirin 150 mg PO
- Requested echocardiography and CXR

17/11/21
- Echo - anterior wall motion abnormality
- CXR-clear

Subjective
- Chest pain mild
- SOB improved

Objective
- Speech – normal
- Repeated ECG-prominent Q waves

Management:
- Continue medications and oxygen
- Diet - low salt, low fat, low carb

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18/11/21

- Condition stable
- ECHO repeated: stable, EF >50%
- Pt requested to go home
- Plan to discharge today

Discharge plan:

- Continue aspirin and clopidogrel for lifetime
- Continue Rovista 20 mg HS and Atacand 10 mg OD
- Regular walk for 15-30 mins/day
- Advice smoking and alcohol cessation
- Encourage DASH diet
- F/U after a week with repeat ECG

Writing Task

Using the information in the case notes, write a discharge letter to the patient's GP, informing him about her condition and further care needs. Address the letter to Mr. Qalab Abbas, GP, Private Clinic, Debarr Road, Anchorage, Alaska, USA.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 7

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a resident doctor working at Modbury Hospital, and one of your patients is being discharged today.

Patient Details:
Name: Ms. Olivia Grace
DOB: 17/09/1988
Marital Status: Divorced
Address: Smart Rd, North Eastern Adelaide 5009, SA
Next of kin: Peter Grace (36, brother)
Date Admitted: 17th Dec 2021 (7th hospital admission since diagnosis)
Date Discharged: 27th Dec 2021

Reason for Admission: Decompensated Schizophrenia

Past medical history:
- Diagnosed with schizophrenia 15/05/2015
- Drug induced movement disorder 08/06/2016

Family history:
- Diabetic mother
- Hypertensive father, died 8 years ago of cardiovascular complication
- Maternal aunt suffers from schizophrenia
- 2 biological brothers
- 1 half sister (maternal)

Social background:
- Got married in 2011
- Divorced in 2012 after diagnosis of schizophrenia
- No social circle
- Limited family support (lives with brother)
- Living on disability allowance
- Heavy smoker (20 cigs daily)

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17/12/2021:
- Brought to emergency in combative state
- Actively hallucinating, paranoid (hostile towards brother and hospital staff)
- Refusal to eat and drink
- Brother informs treatment non-compliance
- Admitted, strict monitoring protocol
- STAT IM Diazepam 10 mg with IM Haloperidol 10 mg,
- IV 0.9% NaCl with 5% dextrose water slow infusion over 24 hours

18/12/2021:
- Slight drowsy, talking irrelevantly, hallucinatory behavior
- Continue refusing oral food
- Started Risperidone 2 mg b.i.d, Procyclidine 5 mg b.i.d (reduce risk of EPS), Lorazepam 1 mg HS

20/12/2021:
- No Improvement
- Increased Risperidone 2 mg t.i.d
- IV fluids continued

22/12/2021:
- Decreased paranoia but reports hallucination
- Ready to eat cake and sips of juice
- IV fluids stopped

25/12/2021:
- Condition improving, no anger, aggression, ideas of persecution, ↓hallucinations
- Eating and drinking well & sleeping well
- Extrapyramidal symptoms (EPS) like muscular rigidity, spasms, tremors - Nil
- Started on fortnightly IM inj. Clopixol depot 200 mg (due to history of noncompliance), tab. Procyclidine 5 mg bid

27/12/2021:
- Condition stage
- Discharged
- Can follow commands but speech still irrelevant
- Denying delusions and hallucinations
- Sleep and appetite fine

Discharge Plan:
- Fortnightly IM inj. Clopixol depot 200 mg (next due on 09/01/2022)
- Tab. Procyclidine 5 mg bid
- Follow up with her psychiatrist
  - rehabilitation
  - psychosocial skills training
  - re-emphasise treatment compliance

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Writing Task

Using the information in the case notes, you need to write a discharge letter to Ms. Olivia Grace's psychiatrist, Dr. Mark Gregory, 127/55 Modbury Rd, North Eastern Adelaide 6007, SA.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 8

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a consultant rheumatologist at Royal Melbourne Hospital, 300 Grattan St, Parkville VIC 3050, Australia. Ms. Bisma Saleem was referred to you by her GP.

Patient details:
Name: Ms. Bisma Saleem
Age: 17 years
Occupation: High School Student
Next of kin: Saleem Merchant (40, father)
Father's occupation: Coal mine engineer
Ethnicity: Asian (immigrant)
Date seen: 14/01/2022

Past medical history:
- Chronic anemia, recognized since menarche at 9 years age, twice blood transfusions (2014, 2016)

Family history:
- Mother: 38, from Bangladesh, suffers from Hashimoto's thyroiditis
- Father: 40, from Pakistan, no known co-morbids
- Parents immigrated to Australia 6 years back
- 1 younger sibling, sister, 6 years old, congenital hypothyroidism

Social factors:
- No social circle
- Does not drink or smoke
- Not sexually active

Past medications:
- Acetaminophen
- Antibiotics
- Antiallergics

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Current appointment (14/01/22)
Presenting complaints:
- Facial rash
- Recurrent oral ulcers
- Chronic low-grade fever
- Weight loss of 4-kg in 2 months
- Fatigue, lethargy
- Frothy, foul-smelling urine
- 2 episodes of tonic-clonic seizures (in last 24 hours)
- Body aches

Examination:
- BP: 97/74 mmHg
- Pulse: 102/ min
- Temp. 100 F
- BMI: 18 kg/m2
- Temporal alopecia
- Butterfly rash on cheeks
- Multiple oral ulcers
- Cold extremities, with bluish distal digits
- Mild PIP, MCP, and wrist joint swelling, tenderness, redness

Laboratory investigation findings:
- Hb 10.3 mg/dl, WBC 7000/mm3, plt 102,000/mm3
- Direct Coombs Test +ve
- Antinuclear antibody (ANA) +ve
- Anti-dsDNA +ve
- Cr 1.3 mg/dl
- Spot protein/spot creatinine ratio 1 g/ day
- WBC casts on urine DR
- MRI plain brain - awaited
- EEG - awaited

Working diagnosis: Systemic lupus erythematosus (SLE)

Plan:
- Hydroxychloroquine 200 mg daily
- Ibuprofen 400 mg TID
- Prednisone 10 mg OD
- Assess clinical response in 7 days
- Arrange arthrocentesis & ECHO
- Need for LP?
- Regular monitoring for electrolytes, seizures
- Inform referring GP
- Counsel family

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Writing Task

Using the information in the case notes, write an update letter to Dr. Marry Elsworth, Ms. Bisma Saleem's GP, practicing at New Century Medical Surgery, 94 Clemency Road, East Melbourne.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Number of Tests	**3**	**10**	**20**
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Reading Parts B & C	✓	✓	✓
Video Course			
Part A Strategies	✗	✗	✓
Part B Strategies	✗	✗	✓
Part C Strategies	✗	✗	✓
Tips and Tricks Ebook	✗	✗	✓
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Case Notes Study 9

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WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a psychiatrist, practising at Mental Health Center, Queensland, and Aron has been your regular patient.

Office: Mental Health Center, Herston QLD 4006, Australia.

Patient Details:
Name: Mr. Aron Steve
DOB: 07/10/1996 (25 yrs)
Marital status: Single
Occupation: Chemistry lab assistant
Education: High school diploma in chemistry
Next of kin: Amy, Steve (47, mother)

Past Medical History:
- Bronchitis (15/09/19)

Past Surgical History:
- Appendectomy (14/05/2012)

Social Background:
- Only child, raised by single mother
- Mother hemiplegic, struggled with heroin addiction
- Father left his mother before his birth
- Moderate smoker
- Consuming 4-5 beer cans daily

Psychiatric Background:
06/07/2002
- Diagnosis: Dyslexia (Sx learning difficulty, problems reading, severe crying spells, isolated behavior)
- Referred to specialized school

17/07/2016
- Brought to clinic by mother with low mood, decreased interest in all activities, social withdrawal,anorexia, insomnia, feelings of guilt and worthlessness, HAM-D score 18
- Diagnosis: Moderate depression with low suicide risk
- Treated as outpatient with Escitalopram 10mg OD, Lorazepam 0.5 mg HS
- Follow up 2 weeks

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18/09/2016
- Remission achieved, HAM-D score 5
- Advised to continue Escitalopram 10 mg for 6 months

19/09/2017
- Brought to emergency with suicidal ideation, anhedonia, decreased attention and concentration
- O/E: suicidal risk high & admitted
- Tab. Escitalopram 10 mg OD (increased to 20mg after 4 days), Alprazolam 0.25 HS
- 28/09/2017: Discharged stable, low suicidal risk. Advised Escitalopram 10 mg OD & regular counselling

16/06/2021
- Presented with repetitive hand washing (20-30 times/day), frequent unavoidable thoughts regarding contamination, severe anxiety, problems at job
- Diagnosed with Obsessive Compulsive Disorder (OCD)
- Advised Tab. Sertraline 25mg OD, follow-up 2 weeks

02/07/2021:
- 1st follow-up, decreased thoughts & frequency but repetitive hand washing persists
- Advised Tab. Sertraline 50 mg OD, follow-up 1 month

Today (02/08/21):
- Obsessions controlled slightly
- Compulsions exist with same intensity
- Severe anxiety
- Reduced functionality at work

Plan:
- Increase Tab. Sertraline to 100 mg OD & Tab. Alprazolam 0.5mg HS
- Referral to psychologist for CBT (evidence of greater benefit with combination CBT and drug treatment)
- Follow up 1 month

Writing Task

Using the information in the case notes, you need to write a referral letter to the psychologist to administer CBT and address psychological conflicts. Address the letter to Ms. Julia Wilson, New Horizon Counselling, 288 Herston Rd, QLD.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 10

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WRITING SUB-TEST: **MEDICINE**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a GP working at Newton Medical Practice and have been managing Ms. Beatrice Angel for the past 2 years.

Today's Date: 21st Dec 2021

Patient Details:
Name: Ms. Beatrice Angel
Age: 60
Marital status: Widow
Next of kin: Elma Wilson (55, sister)

Diagnosis: Type 2 Diabetes Mellitus (poorly controlled)

Medical History:
- Essential hypertension (09/06/2017)
- Hypercholesterolemia (3 years)
- Uncontrolled type II DM (2 years)
- MI (02/10/2020)

Regular Medications:
- Losartan 50 mg o.d
- Atorvastatin 10 mg h.s
- Metformin 500 t.i.d

Social Background:
- Retired clerk, no children
- No social support, Lives in old age home
- Enjoys reading books
- Sedentary lifestyle, BMI 31 (obese, 130 kg)
- Smokes 20 cigarettes per day, Social drinker

Medical Background:
12/11/2019
- Presented with polyphagia, polyuria, fatigue, and hypertension, lethargy.
- FBS 7.2 mmol/l, HbA1c 7%
- Diagnosed with Type II DM
- Advised lifestyle modification, stop smoking, SMBG

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19/11/2019
- Uncontrolled BG at follow up
- Non-compliant to treatment
- Advised Metformin 500 b.i.d.
- Emphasised need for regular medicine intake, lifestyle modification & stop smoking

02/10/2020
- Admitted: MI (ECG ST elevation, HbA1c 6.7%)
- Treatment: PCI, Aspirin 325 mg STAT
- Discharged on 12/10/2020 stable, HbA1c 5.9%

11/12/2021
- Admitted HHS (glucose 30 mmol/L, serum osmolarity 340 mOsm/kg)
- Management: IV fluids, electrolyte replacement, insulin therapy (0.1 units per kg i.v bolus then 0.1 units per kg/hr till bg within range)
- Discharged on 17/12/2021 with stable HbA1c 6.1%
- Refused insulin (↑ Metformin 500 t.i.d)

21/12/2021
- Signs of early Rt foot infection (fever, localised swelling, redness, and pain)
- Still non-compliant - medicine & recommended lifestyle modification

Plan:
- Cephalexin 500mg PO b.i.d, & Paracetamol 1000mg P.O q4hr for fever
- Follow up 1 month
- Referral to endocrinologist for management of uncontrolled BG and related complications

Writing Task

Using the information in the case notes, you need to write a referral letter to the endocrinologist. Address the letter to Mr. Jack Stevens, New Horizon Medical Centre, 288 Herston Rd, QLD.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Writing Practice for
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Case Notes Study 1

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WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Ms. Tracy Ryan, a female patient, shifted to the post anesthesia care unit of James Cook University Hospital, Marton Rd, Middlesbrough TS4 3BW, UK, after having abdominal exploration surgery. You are the PACU nurse. Read the following case notes, and then complete the writing task below.

Patient's info:
Name: Tracy Ryan
DOB: 21-02-1958
Marital Status: Married, 2 daughters
Next of Kin: Husband, 50 yo, Engineer

Family History:
- Mother has rheumatoid arthritis
- father has hypertension & moderate aortic stenosis

Social History:
- Secretary in marketing company
- No drinking or smoking

Medical History: NAD

Surgical hx:
- Appendectomy 3 years ago
- 2 cesarean deliveries

Admission day: 20-09-2021

Dx: Adhesive intestinal obstruction

21-09-2021- Surgery Day

Surgery events:
Pt. had abdominal exploration following clinical & radiological suspicion of intestinal obstruction
- Received general anesthesia, ETT tube & +ve pressure ventilation
- Started laparoscopically but shifted to open after uncontrolled bleeding
- Pt. lost a lot of blood intraoperatively, received 2 units packed RBCs & 1 unit plasma
- Incision midline, closed with staples not sutures

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PACU nursing notes after Pt arrived:

- Vitals stable, BP 125/66 HR 89 Spo2 98
- Administered 1000 ml ringer solution IV
- Upon waking, complained of little pain, doctor ordered 1000 mg paracetamol IV & Ketorolac ampule IM
- Stayed in PACU for 1 hour following surgery
- Fully conscious, talking, will be transferred to general surgery ward
- Pt. has urinary catheter attached to urine bag, central venous catheter for administration of drugs & fluids, 2 drains from abdomen attached to collecting bags

Nursing instructions in ward:

- Monitor patient's vital signs, report to dr. if any hypotension or tachycardia or fever
- Keep incision sterilized & dry
- Monitor drains regularly, alert dr. if saw unusual amount of blood
- CVC must be kept sterilized, change dressing daily
- Urine output should be calculated every 12 hours, if low, report to dr.
- IV ringer solution 10 ml/kg slowly over 24h
- Ceftriaxone 500 IV every 12 h after sensitivity test
- Alert dr. if noticed any respiratory distress
- Continue wearing compression elastic stocking

Writing Task

Using the Information in the case notes above, write a transfer letter to general surgery ward residential nurse, Ms. Daisy Anderson, summarizing surgery notes, and postoperative nursing instructions.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 2

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WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Alex Davies, a male child, was admitted to the pediatric oncology department. You are the pediatric residential nurse. Read the following case notes, and then complete the writing task below.

Patient's info:
Name: Alex Davies
DOB: 01-07-2014, 8yo
Next of Kin: Mother 33 yrs, staying with him in hospital
Family History: Father asthmatic
Social History: Preschool child, lives with his parents, no siblings
Admission day: 11-12-2021
Medical History: Recurrent chest infections, Stunted growth in early childhood
Surgical hx: No surgical hx

12-12-2021
Hospital: Bristol Royal Hospital for Children, Upper Maudlin St, Bristol BS2 8BJ, United Kingdom
Pt. admitted yesterday to pediatric oncology department to complete diagnosis of type of leukemia & start chemotherapy protocol

Patient's medical details:
- Last week severe paleness, paediatrician requested CBC, Hb was 5 gm/dl
- Admitted to hospital, received 1 unit packed RBCs
- Biopsy procedure done yesterday under sedation
- Needle puncture sites in iliac bone & humerus

Dx: Acute lymphocytic leukemia

Nursing management:
- Pt psychologically affected, scared of nurses & needles
- Cries & shouts vigorously if any nurse comes near him
- Trials to familiarize with child, telling stories, offering toys
- Starts to show purpura & petechiae
- Doctor ordered CBC to check platelets, platelet count 6000, doctor ordered 4 units of platelets consecutively
- Pt. started to be feverish, temp. 38°C
- Dressing over biopsy punctures sterilized & changed
- Mother said low urine output, diapers were put on to roughly estimate urine amount

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Nursing instructions for the next shift:

- Familiarize with Pt, he likes being told stories
- Report to dr. if feverish
- Provide healthy diet with vegetables & fruits, also white easily digested protein such as poultry
- Chemo to be given in next few hours, make sure to report any irregular signs to dr.
- Monitor amount of full diapers per day, report to dr.
- Paracetamol suppository every 12 hrs with mother's help
- IV ringer solution 6 ml/kg slowly over 24 h for following day

Writing Task

Using the Information in the case notes above, write an update letter to the paediatric residential nurse in the following shift explaining the patient's condition and further management instructions. Address the letter to Ms. Jackie Wilson.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

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Case Notes Study 3

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Mr. Nagib Smith, a male patient in ICU of Croydon University Hospital, 530 London Rd, Thornton Heath CR7 7YE, UK. You are the ICU residential nurse. Read the following case notes, and then complete the writing task below.

Patient's Info:
Name: Nagib Smith
DOB: 28-06-1952
Marital Status: Married, no kids
Next of Kin: Wife, 52 yo, teacher
Family History: Brother has CRF, mother diabetic
Social History: Plumber, heavy smoker & occasional alcohol intake

Medical History:
- Diabetic, unstable, on biguanides for 3 years
- Dx with hepatoma a month ago, on chemotherapy after liver resection surgery to remove tumor
- Smokes one pack per day
- Tonsillectomy as a child
- Inguinal herniorrhaphy 3 years ago

Admission: 14-01-2022

20-01-2022
Pt. in ICU for 6 days now

Dx: Liver cancer metastasized to both lungs, confirmed by PET CT scan

Patient's Medical Details:
- Has multiple metastatic pulmonary nodules
- Pt. had hypoglycemic coma today
- Consciousness lost, even after raising blood glucose level
- Doctor decided to intubate till pt. gained full consciousness to avoid aspiration

Intubation:
- ETT inserted, pt. GCS 3
- Vitals stable, BP 108/61 HR 100 Spo2 95 after intubation
- Ventilator tube attached for +ve pressure ventilation

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ICU Care:

- Pt. bathed, position changed every 3 hours
- ETT tube suctioned repetitively, pt. has a lot of secretions
- Pt. has urinary catheter, central venous catheter, ETT tube and nasogastric tube

Nursing Management:

- Continue suctioning ETT tube to avoid tube blockage
- Pt. should be 45° up to enhance lung condition
- Report to dr. if any improvement in consciousness level noticed
- Care of diapers, clean thoroughly to avoid bed sores
- Monitor vital signs, report to dr. if any arrhythmia or hypotension occurs
- Calculate UOP as pt. is oliguric, might require dialysis
- IV ringer solution 10 ml/kg slowly over 24h, unless patient becomes anuric

Writing Task

Using the Information in the case notes above, write an update letter to the patient's PCP explaining the patient's condition and nursing management instructions. Address the letter to Dr. Andrea Franklin at 3 Strouts Pl, London E2 7QU, UK.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 4

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WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Ms. Layla Taylor is a female patient in the general surgery ward at Croydon University Hospital, Mayday House, London Road, Thornton Heath, UK. You are the ward residential nurse. Read the following case notes, and then complete the writing task below.

Patient Details:
Name: Layla Taylor
DOB: 21-11-1987
Age: 33 yrs
Marital Status: Divorced, no kids
Next of Kin: Brother, 40 yrs
Family History: Father has third degree heart block, mother has kyphoscoliosis
Social History: Modelling in fashion shows
Medical History: Bronchial asthma & hyperthyroidism
Surgical History: NAD

Admission:
20-11-2021
Pt. arrived at ward after having total thyroidectomy due to clinical, laboratory, radiological diagnosis of multinodular goiter

Surgery Events:
- Surgery performed under general anesthesia, airway easy, no complication
- Goiter huge, lump in front of patient's neck
- Surgery lasted 2 hours, dissection difficult, took time to resect both thyroid lobes, no bleeding
- Incision transverse lower midcervical, about 10 cm long
- Recovered from anesthesia
- In PACU, pt. complained of pain, received paracetamol IV & Nalbuphine 10mg IV
- No complications, pt. transferred to ward

Ward Nursing Instructions:
- Monitor patient closely, be careful of any respiratory complaints
- Blood pressure, pulse, RR & temperature should be measured every hour
- Sterilize incision, change dressing twice per day
- Surgeon inserted one small drain, check regularly for bleeding

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- Encourage oral fluids within 3 hours of surgery
- IV ringer solution 3 ml/kg slowly over 24h
- Amoxicillin Clavulunic acid 500 IV every 12h after sensitivity test

At 6 pm (Same Day)
- Pt. coughing, repetitively
- Nurse prepared bronchodilator inhalation session & notified surgeon for possible asthma
- Cough became worse, turned to abnormal high pitched wheezy sound, stridor
- Pt. struggling to inhale, kept pointing to her neck
- Drains clear, no bleeding

Plan:
- Oxygen mask, to alleviate respiratory distress
- Urgent referral to otolaryngologist for urgent tracheostomy procedure

Writing Task

Using the Information in the case notes above, write an urgent referral letter to the otolaryngologist at the same hospital, explaining case circumstances, and asking for immediate assistance. Address the letter to Dr. Sara Liam.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 5

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WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Ms. Donna Paul is a female resident (for 2 years) in Whitebourne Age Care Home at Burleigh Rd, Frimley, Camberley GU16 7EP, UK. You are the residential nurse. Read the following case notes, and then complete the writing task below.

Patient Details:
Name: Donna Paul
DOB: January 21, 1952
Age: 69 yrs
Marital Status: Widow, 1 son
Next of Kin: Son 30 yrs, owner of car shop
Family History: Brother had stroke
Social History: Retired librarian, not smoker
Son rarely visits or calls. Pt. had huge fight on phone with him begging him to come see her a week ago.

Medical History:
- HTN (2010)
- IHD (2015)
- MI (2021)
- Fallopian tube resection after ectopic pregnancy
- Cesarean delivery

20-01-2022
- Pt. had cardiac catheterization with 2 bare-metal stents insertion on 17/01/2022
- Returned to aged care facility this morning

Nursing Instructions:
- Check HR, BP hourly, notify doctor if BP above 140/90 or if HR above 100
- lasix 20 mg twice daily, Carvedilol 20 mg twice daily, Clopidogrel 75 mg twice daily all PO
- Inquire pt. regularly about chest pain, alert doctor if any
- Encourage ambulation as tolerated
- Advice pt. to take oral fluids hourly & avoid ↑ blood viscosity

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Nursing Notes:

- Pt. looks depressed, didn't want to talk to her favorite nurses
- Preferred staying in bed, unwilling to walk
- Stays awake even late at night

21-01-2022 (8am)

- Pt. shaking, crying, urinated in bed, bloody urine
- Nose bleed stained pillow
- Empty alcohol bottle by her side
- Clopidogrel drug bottle empty
- Nurse talked to pt, trying to calm her, get to know what happened
- Pt. admitted mixing alcohol with drugs, taking overdose of Clopidogrel to get her son to visit her
- Pt. threatened to do it again, shouting at nurse to call her son
- Nurse called EMS summarizing patient's medical condition & incident – urgent transfer to ED decided

Dx: Attempted suicide with overdose of blood thinner medication

Writing Task

Using the Information in the case notes above, write an urgent referral letter to emergency physician Dr. Ahmed Nour at Frimley Park Hospital, Portsmouth Rd, Frimley, Camberley GU16 7UJ, United Kingdom, requesting the patient's medical and psychological management.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 6

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are the ward residential nurse at Gartnavel General Hospital, 1053 Great Western Rd, Glasgow G12 0YN, UK.Ms. Scarlet Wilson, a female patient was admitted in the vascular surgery. Read the following case notes, and then complete the writing task below.

Patient Details:
Name: Scarlet Wilson
DOB: 01-04-1989
Marital Status: Married, 3 kids
Next of Kin: Husband, 36 yrs, Lawyer
Family History: Mother had varicose veins, father has Behcet's disease
Social History: Stay at home mother

Medical History:
- Preeclampsia in last pregnancy
- Varicose veins, failed medical treatments
- DVT 6 months ago, on blood thinner (warfarin)

Surgical History: 3 cesarean deliveries, last 8 months ago

Date of Admission
26-01-2022: Surgery same day

Surgery Events:
- Pt. had stripping of varicose vein
- Surgery done under spinal anesthesia
- Lasted 90 min.
- Pt. stopped taking warfarin 1 week before surgery, doctor expected bleeding, didn't happen
- Uneventful, no complications
- Pt. transferred to PACU then ward

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Ward Nursing Management:

- Pt. has high risk for DVT & pulmonary embolism, monitor vitals, awareness level & if chest pain occurred
- Hydrated pt, 10 ml/kg IV fluids per day + oral fluid intake
- PCA connected for analgesia over next 24 hours
- Dressing changed once on night of surgery, wound sterilized with betadine
- Pt. recovering well, no visible complications, minimal risk
- Discharge planned 27-01-2022

Discharge Plan:

- Advice pt. to get enough rest & sleep
- Wear tight bandage first 4 days post surgery
- No bathing for first week, keep tight bandage dry & clean
- Ketorolac IM injection/ 12 h for first 3 days
- Try walking slowly at day 6 post surgery, stop walking if severe pain or bleeding
- Resume Warfarin 3 days after surgery, to avoid bleeding risk
- Legs elevated on pillow above heart level, as many hours as possible to reduce bruising
- Usual diet, protein to enhance recovery, fresh fruits & vegetables
- Return to dr. after 7 days, for incision recheck & medications adjustment

Writing Task

Using the Information in the case notes above, write a discharge letter to home care nurse Ms. Alberta Allen, summarizing surgery notes, and providing home nursing instructions.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 7

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Tom Johnson, a male child, was admitted to the pediatric surgery department at Royal Manchester Children's Hospital, Oxford Rd, Manchester M13 9WL, UK. You are the pediatric residential nurse. Read the following case notes, and then complete the writing task below.

Patient's Info:
Name: Tom Johnson
DOB: 04-10-2021
Next of Kin: Mother (22 yrs)
Family History: Mother has DM type 1 (on insulin)
Admission Day: 10-02-2022
Medical hx: NAD

Diagnosis: Bilateral undescended testicles, scheduled for bilateral orchiopexy surgery

10-02-2022
Admitted to hospital with his mother, scheduled to have surgery next day

Patient's Medical Details:
- Healthy breastfed 4 month old baby
- Condition diagnosed by baby's pediatrician on regular checkup
- Testicles can be felt in baby's abdomen on palpation
- Xray & MRI abdomen done to confirm presence & site of testicles
- Scheduled operation to preserve future fertility & avoid cancer risk

11-02-2022 (Surgery day)
- Mother withheld breast milk for 4 hours to complete pre anesthesia fasting period
- Baby subjected to general anesthesia with spontaneous ventilation
- 2 bilateral small groin incisions made, surgery lasted 80 min.
- Baby awakened, no complications, shifted to PACU then to ward

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Ward Nursing Management:

- Baby should resume breastfeeding 2 hours after end of surgery
- Incision care: Keep dry & sterilized, change dressing regularly as incision can get contaminated with baby's stool or urine
- Care of mother's blood sugar insulin intake, as breast feeding might cause hypoglycemia which would exaggerate if taking regular insulin
- Give Amoxicillin- Clavulanic 250 mg twice daily IV
- Paracetamol oral suspension, 2 ml every 8h for pain
- Discharge planned 12-02-2022
- R/V 7 days for incision checking

Nursing Instructions Post Discharge:

- Change dressing as soon as it gets wet, teach mother how to do it, use betadine
- Support mother's breastfeeding, teach her different breast-feeding positions
- Encourage mother to eat healthy nutritious food to ensure quality breast milk for baby
- Keep an eye on mother's medical condition, advice regular blood glucose checks, teach possible hypoglycemia signs
- Instruct mother to monitor baby's temperature
- Keep baby hydrated, make sure diapers filled with urine

Writing Task

Using the Information in the case notes above, write a discharge letter to the pediatric community health nurse explaining the patient's & mother's medical condition & nursing management instructions. Address the letter to Ms. Lora Wilson.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 8

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
Mr. Damian Barnes , a male patient in the ICU of Royal Hospital For Neuro-Disabilities, West Hill, London SW15 3SW, UK, where you are the ICU residential nurse.

Patient's Info:
Name: Damian Barnes
DOB: 28-06-1962
Marital Status: Widowed, 2 sons
Next of Kin: Son, 30 yrs, lab. technician
Family History: Father died of hemorrhagic stroke, brother ankylosing spondylitis
Social History: Teacher, smoker (10 cig/d)

Medical History:
- HTN since 2013
- Type 2 DM since 2010
- Dyslipedemia & atherosclerosis

Surgical hx:
- Lt. diabetic foot amputation 2018
- Inguinal herniorrhaphy 2005

04-01-2022
Pt. admitted to ICU 11 days ago for malignant hypertension control, developed massive hemorrhagic stroke afterwards

Medical Details:
- On admission, pt. fully conscious, aware to time, place & person
- BP 190/120 HR 100
- Pt. developed pulmonary edema, dr. started lasix infusion
- Pt. received nitroprusside sodium IV infusion at rate 10 mcg/kg/min
- Pt. Confused, stuttering, disturbed consciousness level
- CT brain showed massive hemorrhagic stroke
- Neurosurgery consulted, no benefit from surgery, patient's condition won't improve
- Pt. Intubated & ventilated, GCS 4

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16-01-2022

- Pt. transferred to OR for scheduled tracheostomy operation as pt. expected to stay ventilated in foreseeable future
- Operation done, pt. returned to ICU

ICU Nursing instructions:

- Urgently page ENT surgeon if noticed any blood in or around tracheostomy tube
- Suction via tracheostomy tube carefully, to avoid injury & bleeding
- Monitor patient's respiration & ventilation, if any abnormality, tracheostomy tube to be unsealed
- Provide hourly nasogastric tube nutrition, rest 8 hours at night
- Head elevation 45° up to decrease possible rebleeding
- BP should be maintained b/w 120/60 & 150/70
- Regularly change patient's position, to avoid bed sores
- Check bl. glucose level hourly, stop insulin infusion if blood glucose less than 300
- Monitor pt. consciousness level, notify doctor with any changes
- Check UOP every 12h, check urine catheter for color of urine
- Pt. has recently inserted CVC catheter, change dressing & sterilize every 12h
- Fluids, IV ringer solution 12 ml/kg slowly over 24h

Writing Task

Using the Information in the case notes above, write an update letter to the patient's community health nurse explaining the patient's current condition and nursing management. Address the letter to Ms. Nouran Ali at St Magnus House, Lower Thames St, London EC3R 6HD, UK.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 9

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a student health nurse at Oswald Community College, Texas, USA.

Today's date: 25th January 2012
Name: Nathalie Wiggins
Age: 24 years
Gender: Female
Diagnosis: Suicidal ideation
Place of birth: Haiti
Status: Nursing College student (yr. 1)

Social History:
- Immigrated to US-December 2012 (student visa)
- Has few friends
- Introvert nature
- Victim of racial discrimination
- Lives in university hostel (off campus)
- Shares room with 3 people
- Part-time waitress (local café)

Educational History:
- Joined nursing course by force (parental pressure)
- Interests: fashion designing
- Last CGPA 2.0 (failed few subjects)
- Flunked the mid-semester OSCE
- Can't concentrate on studies

Family History:
- Eldest child (has 3 younger brothers)
- Paternal aunts- Depression
- Grandmother-Anxiety disorder
- Grandfather-committed suicide

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Current Medical History
14th July 2012-10:00

- Received at the clinic after exam
- C/O fear of failure in the exam
- Crying, anxious, fearful, paranoid
- Cannot go to the job (taken leaves for 15 days)

Advised: Stress management techniques

25th July 2012-9:00 pm

- Received in clinic
- Accompanied by roommate-found her with rat killer
- C/O severe distress, desire to kill herself (failed in final exams)
- Can't manage stress
- Smokes cocaine cigarettes (excessive)
- Hasn't alked to family - past 3 months
- Bunks university classes
- Lacerations on B/L arms (blade cuts)

Plan

- Arrange immediate psychiatric management
- Inform parents

Writing Task:

Using the information given in the case notes, write an urgent referral letter to Dr. Lamis Beshir, Psychiatrist and Mental Health Counsellor, Oswald psychiatric hospital, Texas, USA, requesting emergency psychiatric management.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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OET Reading

	Basic	Intermediate	Comprehensive *Most popular*
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Reading Part A	✓	✓	✓
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Part A Strategies	✗	✗	✓
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Part C Strategies	✗	✗	✓
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	Trial	Standard	Intensive *Most popular*
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Validity	1 week	1 month	3 months
Price	**$10**	**$25**	**$50**
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Case Notes Study 10

Benchmark OET Writing Correction Service

WRITING SUB-TEST: **NURSING**
TIME ALLOWED: **READING TIME: 5 MINUTES**
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a nurse in charge at Bloomberg Home Age Care agency staying with the patient during his hospital admission. The patient is now shifting back to the old age care home.

Today's date: 16th November 2021
Name: Giuseppe Matteo
Gender: Male
DOB: 25/08/1950
Age: 71 years
DOA: 15th November 2012
Diagnosis: End-Stage Renal Disease (ESRD), mild dementia, Rt Lobar pneumonia
Place of birth: Italy

Social History:
- Lives at the old age care home
- Difficulty speaking English
- Wife-Maria (64 years)-lives in her own apartment
- One son, Giovanni, a plumber (occasionally visits)

Medical History:
- H/O ESRD 5 years
- Dialysis (twice a week)
- Frequent breathing issues-on 2L oxygen (NP)
- Medication non-compliance

Family History:
- T2DM-mother and sister
- CRF caused father's death
- Death of younger sister-RTA
- Death of younger brother-Suicide
- Rheumatoid arthritis-wife (9 years)

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Recent Nursing Notes

15th November 2021 (6:00 pm)

- Arrived at an emergency-Resuscitation unit from old age care center
- Severe SOB and agitation
- GCS 13/15
- BP 160/100 mm Hg, HR 110 b/min, T 36.8 C, O2Sat 81%
- ABGs stat-revealed hypercapnia-respiratory acidosis
- ECG done-sinus tachycardia
- Pt kept CBR
- Foley catheter inserted
- Dialysis begun
- Intubation done-Mechanical ventilator attached (full support)
- Pt is sedated and relaxed
- Shifted to ICU

16th November 2021 (6:35 am)

- ICU physician rounds done
- ABGs improved
- Extubated and breathing via BIPAP
- NG feed 10 ml/h
- Pt is discharged today

Discharge Plan

- Shift pt. back to old age home
- Arrange Oxygen therapy and BIPAP machine
- NG feed (Glucerna) 100 ml q6h
- BGL premeal
- Dialysis (twice a week)
- Involve physiotherapist (chest physiotherapy)
- ROM q4h
- Positioning and back care q2h to avoid pressure ulcers
- Toileting assistance
- Hygiene care

Writing Task

Using the information given in the case notes, write a discharge letter to Ms. Shenaz Abraham, home care supervisor, updating her about patient's condition and home care requirements. Address the letter to Ms. Shenaz Abraham, Bloomberg Home care agency, Chapel Hill, NC.

- **DO NOT use note form in the letter.**
- **Expand the case notes where relevant into full sentences.**
- **Use formal letter format.**

The main part of the letter should be approximately 180-200 words long.

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Case Notes Study 1

WRITING SUB-TEST: MEDICINE
TIME ALLOWED: READING TIME: 5 MINUTES
WRITING TIME: 40 MINUTES

Read the case notes and complete the writing task which follows.

Notes:
You are a GP working at Newton Medical Practice and have been managing Ms. Beatrice Angel for the past 2 years for multiple medical conditions.

Office: Newton Medical Practice, 16-18 Newton Rd, Melbourne.
Today's Date: 21st Dec 2021

Patient Details:
Name: Ms. Beatrice Angel
Age: 60
Marital status: Widow
Next of kin: Elma Wilson (55, sister)

Diagnosis: Type 2 Diabetes Mellitus, IHD

Past Medical History:
- Essential hypertension (09/06/2017), Losartan 50 mg o.d
- Hypercholesterolemia (3 years), Atorvastatin 10 mg h.s
- Uncontrolled type II DM (2 years), Metformin 500 t.i.d
- MI (02/10/2020), Aspirin 300 mg o.d

Social Background:
- Retired clerk, lives in old age home
- Adopted stray dog 2 years back
- 2-3 friends, not close enough
- No children
- Sedentary lifestyle, BMI 31 (obese, 130 kg)
- Social drinker
- Smoker (20 cigs daily- 15 years)

Medical Background:
12/11/2019
Presented with polyphagia, polyuria, fatigue, and hypertension, lethargy. FBS 7.2 mmol/l, HbA1c 7%, TC 250 mg/dl
Diagnosed with Type II DM
Advised lifestyle modification, SMBG

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Uncontrolled BG at follow ups (19/11/2019), (02/12/2019), (10/01/2020)
Non-compliant to treatment
Advised Metformin 500 b.i.d.
Emphasised need for regular medicine intake, lifestyle modification, stop smoking

04/04/2020
Admitted: moderate depressive episode, BG still high.
Given counseling
Refused drug treatment
Referral to clinical psychologist
Discharged on 06/04/2020: LAMA

02/10/2020
Admitted: MI (ECG ST elevation, HbA1c 6.7%)
Treatment: PCI, Aspirin 325 mg STAT
Discharged on 12/10/2020 stable, HbA1c 5.9%

11/12/2021
Admitted HHS (glucose 30 mmol/L, serum osmolarity 340 mOsm/kg) IV fluids, electrolyte replacement, insulin therapy (0.1 units per kg i.v bolus then 0.1 units per kg/hr till bg within range)
Discharged on 17/12/2021 stable HbA1c 6.1%,
Discharge meds: Metformin 500 t.i.d (refused insulin), losartan 50 mg o.d, atorvastatin 10 mg h.s, Aspirin 300 mg o.d

21/12/2021
- Signs of early Rt foot infection (fever, localised swelling, redness, and pain)
- Still non-compliant medicine & recommended lifestyle modification

Plan:
- Abx treatment for foot infection, Tab. Cephalexin 500mg PO b.i.d, Tab. Paracetamol 1000mg P.O q4hr for fever.
- Follow up 1 month
- Referral to endocrinologist for management of uncontrolled BG and related complications

Writing Task

Using the information in the case notes, you need to write a referral letter to the endocrinologist. Address the letter to Mr. Jack Stevens, New Horizon Medical Centre, 288 Herston Rd, QLD.

- DO NOT use note form in the letter.
- Expand the case notes where relevant into full sentences.
- Use formal letter format.

The main part of the letter should be approximately 180-200 words long.

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